SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Avelumab — metastatic Merkel-cell carcinoma

Metastatic Merkel-cell carcinoma after chemotherapy · NCCP 00535 · version 7 · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
To help immune cells control metastatic Merkel-cell cancer after chemotherapy; benefit, side effects and suitability for continued treatment are reviewed regularly.

Schedule
Avelumab by IV infusion every 14 days, continuing while beneficial and tolerable. The first four infusions have a specific premedication plan; four is not the course limit.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Avelumab
PD-L1 CHECKPOINT IMMUNOTHERAPY

- ☐ Infusion reactions
- ☐ Bowel, lung, liver or kidney inflammation
- ☐ Thyroid, adrenal, pituitary and glucose problems
- ☐ Skin reactions
- ☐ Rare heart, nerve, muscle or eye inflammation

Discuss the exact diagnosis and treatment intent, expected benefit and uncertainty, alternatives, regimen-specific risks, tests, fertility, practical burdens and your preferences. This guide supports the consent conversation and does not replace it. This is the Merkel-cell cancer context after chemotherapy under current NCCP. It is not avelumab maintenance for urothelial cancer. Evidence from untreated first-line Merkel-cell cohorts must not be presented as the evidence for this specific post-chemotherapy indication.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00535 v7.

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